AI | Expert Insights

COMPANY DEEP-DIVE

Vice President of Operations at Health Prime Believes *Agentic AI* Enhances Efficiency in *Healthcare Revenue Cycle* Management

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06 Sep 2026	08 Jun 2026	Consultant	Business-Led	EPIC SYSTEMS

SMART SUMMARY

Summary

The conversation focused on the practical applications, benefits, and limitations of ***agentic AI*** in ***healthcare revenue cycle*** management, including its impact on cost, workflow integration, and market competition. The expert discussed real-world use cases, implementation challenges, and strategies for integrating AI with traditional BPO models.

EXPERT BACKGROUND

- Vice President of Operations with direct experience implementing and managing ***agentic AI*** solutions across the ***healthcare revenue cycle*** management (RCM) value chain, including call center, accounts receivable (AR), claim scrubbing, and patient collections.

EXPERT SENTIMENT

- Agentic AI delivers meaningful value in automating simple, repetitive tasks in RCM, particularly in call centers and payer communications, but always requires a human in the loop for oversight and complex cases.
- There is **skepticism about vendor claims of dramatic workforce reductions and easy integrations**; the expert believes such promises are often overhyped and not reflective of real-world implementation challenges.
- **The expert sees AI as a tool for incremental cost savings and efficiency**, but not as a replacement for experienced staff or end-to-end automation.

TAKEAWAYS

- **Agentic AI has been successfully implemented in call centers for handling simple patient requests, resulting in a 20% reduction in call volume during peak seasons**.
- In AR, agentic AI is effective for automating payer calls and prior authorization processes, but always with human oversight for complex or clinical tasks.
- Claim scrubbing with AI is effective, especially when the RCM provider owns the end-to-end process, allowing for feedback loops from denials to improve front-end processes.
- Cost improvements from agentic AI are typically in the low double digits, with higher savings possible in areas with more low-hanging fruit, but claims of 20%+ savings are viewed skeptically.
- **The market is seeing increased competition from AI-first RCM companies, but the expert believes deep specialty experience and proven results are key differentiators, with some clients returning after unsuccessful AI-first engagements**.

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Expert Bio

Vice President Operations at Health Prime. The expert is responsible for overseeing the entire front-end, middle, and end-to-end revenue cycle. The expert is responsible for customer service, patient billing, early-out collections, backend and frontend revenue cycle operations, patient, hospital, and physician scheduling, order management and transcription, and self-pay functions including public benefits and omnichannel customer service. The expert is responsible for middle-revenue-cycle functions including physician and acute accounts receivable and clinical denials. The expert is responsible for end-to-end revenue performance and optimization across the full revenue lifecycle. Prior to Health Prime, the expert was the Senior Director of Operations at R1 RCM, leaving June 2024. The expert was responsible for the front-end shared services model. The expert was responsible for customer service, customer access, scheduling, and

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early-out functions delivered both locally and offshore through R1. The expert was responsible for supporting multiple clients and acquisitions across the United States. The expert can speak to RCM and the end-to-end revenue cycle.

Interview Transcript

Client 00:00:00

Thank you for taking the time to speak with us today. We're looking at the use of AI agents in RCM. We were impressed by your background and wanted to see if this is topic that you would be able to speak about.

Expert 00:00:13

Definitely. I've implemented and are using currently AI agents in the entire RCM space in healthcare.

Client 00:00:21

Fantastic. Really, our intent is to be able to understand where in the RCM space are agentic AI use cases being successful and delivering value today versus what use cases are, call it, more hype than reality. Perhaps just walk us through your experience with the RCM value chain end to end and talk about where agentic AI is being implemented, and of those use cases, which ones are truly delivering meaningful value today versus where it's still hype versus reality.

Expert 00:00:51

Absolutely. I think we first got to separate this into two paths, just for the conversation. There's the agentic AI in the call center space, and then there's the agentic AI for the AR space. Obviously, the AR, we can talk about everything. Denials and cash and everything else that gets involved in that. The most important thing when we talk about the use of any type of agentic AI in the call center space, let's just do that one for right now, where it's been used very successfully, and folks like myself absolutely love, is the easy questions, easy requests.

Example, if a patient calls in and says, "I just want a copy of my itemized statement," there is really no need for a human intervention on that, as long as the patient has the information needed. Those, we have rolled out successfully. In peak season, peak season being the January to March for tax season purposes, in most customer service billing departments, we were able to reduce those calls by 20%, **which is a huge impact on the overall of a call center space**. That's one example where you could use it **that has been successful**. We have seen even some of the elderly population more inclined to adapt to it. We have found now that the easier you make it for them, the better it becomes.

One of the things that we have implemented that I always advise folks to do is you have to think about this as, if you want this to work really well, your workflows have to be extremely updated. Not to the process of an agent, but to the process of the mindset of the caller. That's where I have identified that **the capabilities and the actual planning and the positive effect of it** have come. Where it becomes a hype, where we see a lot of even the commercials from Sam Altman and OpenAI, and they are like, "We will reduce your workforce by 80% or 100%." It's impossible. That's nonsense. There is always a human in the loop that must remain. I think that's where the hype becomes.

The other hype is how much they say it's easy to do. While it has gotten better in the last two weeks, I always tell people, "AI changes so frequently that what I was doing two weeks ago, it's now changed. Just slightly, but it has changed enough, and it might be easier." The biggest hype is you get these folks that come in and promise you an easy integration, promise you a fast integration. That's never the case because of the integration part, going back to if your workflows are not good, **it's going to be very hard** for you to get your money worth out of it.

That's where I think and have seen and **I fell into that trap** about two years ago, where we tried to implement an agentic AI in our call center piece, and man, it took us probably six months to even get it working at a point where we were like, "Okay. This works now." It took a long time. I think that's one big hype on it.

Now, in the AR space, talking about AR spaces, one of the greatest integrations, again, going back to this agentic AI, is where **agents waste a ton of time is getting ahold of the payers**. When there's **a complex denial** and they have to call the payer, especially if it's one that doesn't have anything online, and you've already tried, and of course, not updated, or it happens to be that we get the websites that go down every now and then, that happens.

Calling the payers has been our greatest usage of it, where **we've seen success** where our workflows, again, go back to saying, "Okay, if this requires a phone call, do not give this case to an agent. Put it on the agentic AI, have them call the payer, and once it gets a hold of somebody, then connect them to an agent immediately, with the conversation on what took place, the claim number, so it's easy for the agent to use it." The other piece is, again, going back to the workflows, that's where **we've seen a good usage of it** that people can help. **It becomes almost an AI battle** because the payers also have an AI now answering the calls. You have a robot talking to a robot, and it's hilarious if you think about the whole thing. That's one that I've seen work.

Now, where we get hype from the ARs, I get so many people to try to come in and try to sell us what I call snake oil. They're like, "We can reduce your denial rate by X." Sure. You can reduce them, but that piece becomes uncontrollable on us, on the provider side. It's the payer.

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If they forgot to change or didn't change anything on their end, **we're going to get a denial** regardless of if we did everything right. I think a lot of hype comes into them trying to sell you an insane amount of reduction on your denials or things like that. I've seen so many people trying to pitch that sale just to try to get in the door. Again, going back to your workflows and how this is set up, **we get too much of a hype from people on it**.

Prior authorization is a good one. that **we've seen good success on the agentic AI** because we can almost automate almost all the big payers now because their websites rarely change, the access to them rarely changes, but we've always had the human in the loop. We reduce our workforce, but it now allows us to focus more on actual prior authorization, like clinical notes that need to be submitted with the proper documentation that require human intervention. We've seen success there.

Client 00:06:42

What about claim scrubbing? Is that something **you've seen success** with as well? Like you just said, it doesn't require IVR or the call center piece.

Expert 00:06:53

Correct. That claim scrubbing is always a good one. Even Epic has its own claim scrubbing now built into Epic. We've seen it in some of the hospitals that pay for that additional, because Epic will charge you for everything extra.

Client 00:07:07

In claim scrubbing, is there a way to or do the AI-enabled tools do this today, wherein they look at the denials that are coming in and use some of the learning from those denials and bake it into the front end through claim scrubbing, etc.? Is that feedback loop something that's established today?

Expert 00:07:26

Yes, and no. That comes down to if you own the end-to-end revenue, then yes, we have it built in. Where it becomes an issue is where we have physicians and acute AR process for the denials part, but we don't have the pre-registration, prior authorization, and the whole patient access part of it. Let's say that they have a different vendor for that. That's where it becomes really hard.

Client 00:07:54

Where it's the same vendor, you think that piece is working today, the feedback loop, in an agentic way, where learning from denials is then informing not just coding, but perhaps claim scrubbing?

Expert 00:08:08

Yes, it is. That's the beautiful piece of the AI, because once you have it built that way, you can start learning from it and catching trends that we can then raise a flag sooner like, "Hey, we're seeing a lot of X, Y, and Z issues on this claim specifically." They root cause and they say "From prior issues, this is what was happening." It does notify that team. Again, human in the loop. There's always somebody there that checks against it, and then they have the conversation.

Client 00:08:42

Okay. Given these enhancements, I hear you there, **some of it's real and meaningful and is helpful in reducing the time spent, how much of cost improvements** are you seeing from RCM providers, especially AI-enabled? Have **you seen any pressure in RCM pricing given that meaningful aspects of that value chain** can now be automated through AI?

Expert 00:09:03

That's the everyday question that everybody is navigating, us including, in this aspect of things. What we have seen and what **I have noticed just from us is that the savings that come out of the AI and workforce reduction, for one, if I am able to reduce my workforce by 30%**, when we go to the clients, the way we pitch it to them is, obviously, the more revenue that we can get, the cleaner claims that we can get, the sooner we can get the AR in the door, the reduction in AR days.

We are now reducing, obviously, the AR days, bringing in more cash, those kinds of things, where our pricing model, if it's FTE-based, then **we take the savings**, if it's AR-based or collection-based, then we make more because they are making more. Where we are coming to negotiations now, especially with the bigger clients, is more so for them to know, "Hey, yes, we are, but there is the cost to implement AI, where we have to see the benefit of it before we can pass the savings to them."

What this allows us is a longer contracting with them. Where they can now come to us and say, "Hey, let's move from a year-to-year

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contract to a three-year, five-year," which allows us to plan for more investments. If I know I have a client set for five years, it's easier for me to make a decision on whether I want to invest more time into this.

Client 00:10:25

Maybe forget pricing. Maybe **what kinds of improvements in your internal costs** are you seeing from the use of agentic AI today? Are **you seeing cost improvements**, 15%-20%, 30% or more?

Expert 00:10:39

That's a good one. I can give you the average. We can use Q1 of 2026 and then Q4 of 2025. About a low-double digit increase that we have been able to gain. In some, a lot larger, but that's because there is always low-hanging fruit. That skews the data. That's not something that I wouldn't tell people. There are opportunities, but they're so small. In your average, when you really look at this, just using the last two quarters, about low-double digits that we've seen. In some areas, lower than that. If, again, implementation took longer, or the workflows needed to be cleaned a lot more, we still have more cleanup to do before we see that 20%. That's where you want to land. If anybody sells you anything above that, you can pretty much scrub it and say, "Yes, I don't think so."

Client 00:11:36

Makes sense. Just more out of curiosity, what kind of pricing contracts dominate today? Are you seeing more of time and effort, or are you seeing more of percentage of claims? What is the trend for pricing contracts these days?

Expert 00:11:51

The implementation to what we call go-live on it plays a lot into it. We have seen contracts where they are wanting large payments upfront, and we have learned so much from this. They say, "No, we will get this pricing on this contract." We will obviously pay for implementation at the timeline that we are asking and we are demanding. If we don't go live, then **it becomes a penalty** that they pay or delay on payment on it. I can give you examples of different areas where we get people coming in saying, "We can get this done X amount of time, and we've seen this reduction," but they don't know the data integration.

They don't know if it's really going to integrate with Cerner the way they said or with Meditech the way they thought about it, or athenahealth or if Epic doesn't allow it. Epic is trying to control the market as much as they can, so they don't allow integrations of AI outside of what they're already working on. When we come to these contract prices, one of the biggest things we tell them is, "When will we go live? Is the pricing here determined on a cost per usage base? Is it an implementation? Is it after go-live?"

The pricing, you guys really need to focus on. If you know your workflows are decent enough that you can roll this out within the 30-, 60-day period, then the contract pricing becomes easier to do. If you don't know the data integration, then I would say you guys have to be more strict on, "This becomes a contract based on when we go live."

Client 00:13:23

Got it. What is your preferred model when you go out to work with provider groups? Do you try and provide the end-to-end RCM service, or do you also work with taking over parts of the value chain and not the entire value chain in its entirety, for example?

Expert 00:13:38

The answer to that depends on the size of the organization. The way I say that's because if you take a large hospital, like Intermountain Health, for example, you want the end-to-end. Let's say your specialty is anesthesiologist, and you're only going after anesthesiology physicians and services, then that's all you want to do. You don't want to go in and try to get the whole scope of that physician practice is because, especially on the physician side, **it's low dollar margins**, but much, much higher volumes, whereas in the hospital base, it's larger but fewer ones. My preferred model is always, if it's a large organization, go end-to-end.

Client 00:14:19

Got it. End-to-end means starting with coding especially, starting with coding or even more upfront?

Expert 00:14:26

Coding is always a good one to get in if you can get in the door.

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Client 00:14:31

What about collecting patient payments? How do you think of that? What is your stance on that?

Expert 00:14:37

The cost to collect, most folks already know if you are having your call center work being done out of India, then you know what your margins will be. That's the first thing you want to ask, "Can this work be done offshore?" That will determine whether you want to go after the patient collection side. Right now, I would even argue, if you switch to agentic AI on cash collections, you probably would get a better return, especially with all the automation, and there is via text message, phones, email. Heck, a lot of these industries now are texting you. You can tell they're legit by the number they're coming from. It comes to the same one as in your statement. They've learned to do that. Collections, if you can get in the game and get the agentic AI automation place in there, **it's a nice opportunity** right now.

Client 00:15:28

By the way, generally, I'm just curious, keeping agentic AI aside for a second, but how much are you able to collect from your patient payments side, or how much do physicians leave on the table in terms of their patient collections?

Expert 00:15:42

Are you referring to the area for the claims or post? Are you talking balance collections?

Client 00:15:48

The patient responsibility. How much of the patient responsibility is a best-in-class organization usually typically able to collect? Are you able to collect 95%-100%, or are you able to collect only 70%, 80% of the patient responsibility?

Expert 00:16:08

Ideally, you're never going to collect 100%. If anybody comes to you and tells you they collect 100%, I'd be very surprised. I would say the benchmark for that is you want to be within a 90%-92% collection. The 8% are going to be your own collectibles. These are folks who are just never going to pay. It doesn't matter. **You may be knocking their door**, they're just not going to pay.

If you look at the age and population model, we've seen **it successful in certain areas** because of the age population being younger, where we make it easier with AI, versus the elderly population that still write checks. It really depends on that. If you can stay in the 90%, you got a good system in place. **We've seen bad collections down in the 67%**, 72%. 76%. If you were able to collect that, I think you're still making profit, but your margins are in the teens.

Client 00:17:15

Makes sense. Let me ask you this, coming back to the agentic AI piece, as you're bringing on more and more agentic AI solutions, you're still working with your traditional BPO model as well, how do you manage that friction, if you will, between teams who are doing this work manually today, and then you're bringing on agentic solutions? How does that play out?

Expert 00:17:39

What I have found out works very well, and this is my secret sauce that I'm passing to you, is if you reach out to your staff and you say, "Hey, I got this opportunity. It will require time. It will require you to do additional work, but you will learn about AI, and we'll teach you about AI." You send out a survey. You only take the people that are willing to commit to do the work because they still got to do their regular job, but they're also going to help with learning all of this.

Those are the folks that you want to do, and those are the folks you want to bring in into a model. Why? They're the ones who are willing to and wanting to learn about AI. They're probably the ones who have seen enough hype out in the market to say, "This is where the industry is going, and I need to get myself in there." That way, you don't have friction, because they've already volunteered, they already know, and you're going to get their input really good. Once you get those folks to adapt and to see, it's like your proof of concept, then it's easier to get the rest of the team.

These guys have been in the BPO industry for 10, 15 years. They will only take on the additional work if somebody else is already doing it. **That has been a great progress** for us to integrate this instead of going and saying, "Hey, we are going to integrate this automation. You got to do it." Nobody is going to do it. They are just going to fight it. By doing it the way I explained it to you, getting a group of people in to buy in, they get to learn, and then they become your human in the loop. They are the ones who, at the end you can say, "Yes, we reduced workforce by 30%. These are the folks I got to keep." It really just makes it so much easier at the end.

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Client 00:19:29

Makes sense. Let me ask you, in the market, you come across the new breed of RCM companies now who are AI first, completely new companies, their operating model is AI first, and you're competing against them in the market, how do you compete against them? Are you finding practitioner groups leaning towards them? What do they come with? They come with better pricing, for example. How do you compete against them?

Expert 00:19:54

The market has been flooded with a lot of AI-first small companies. They're just trying to grab market share. The biggest differentiator is going to be your specialties, your experienced folks that you bring into the table. I think at the discovery session, when you get with a client, then you submit your RFP, you can list out there, say, "We have 20, 25 years of coding experience in this specialty, like home health." You can bring an AI and do some of that, but you're never going to have the experience on that. The pricing matters a lot because a lot of these AI people come in trying to say, "We will do it for $0.05."

You're going to lose some market share to that. You got to understand it. You're going to lose some of it. We've seen a lot of boomerang clients of ours come back and say, "Hey, we messed up. We need to bring the work back." You use it as a reference case for the next RFP to say "Hey, we've worked with so-and-so who tried an AI. We have AI. We're not AI first." I think that helps. That's how the market is shifting anyway.

Client 00:21:07

Thank you for taking the time to speak with me. Have a nice day.

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